

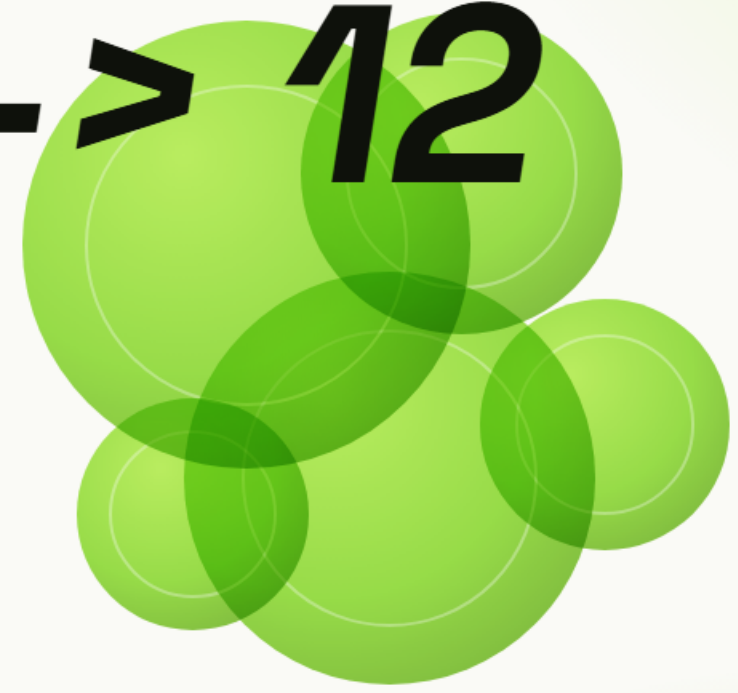


KLINIK PERGIGIAN DATO KERAMAT

EARLY CHILDHOOD CARRIES

68 - > 12

LOW ATTENTION



Ringkasan CPG untuk tindakan klinik

ECC boleh dicegah lebih awal.



Friendly, structured and
evidence-led.

AUDIENCE

KKIA / IM / Dental Team

REGISTER

Tataterbit kerajaan

FOCUS

Risk, prevention, action

SETTING

Public dental clinic

Satu Aliran

Risiko kepada tindakan susulan.

Isi CPG diringkaskan supaya mudah diingat dan digunakan.

01

Kenapa ECC

BURDEN

02

重点

Definisi

CASE LENS

03

Risiko

PATTERN

04

Semakan

ASSESSMENT

05

Pencegahan

PREVENTION

06

Pendidikan

OHE

07

Rawatan

CARE OPTIONS

08

Tindakan

CLINIC FLOW

- READ LESS, ACT CLEARER



Beban Masih Tinggi

Trend turun, kadar masih besar.

Trend turun, kadar masih besar.

71.2%

NOHPS 2015

2005

76.2 % prevalence

2015

4.8 dft severity

Age

<6 years

— Prevalens turun dari 76.2% pada 2005.

- Prevention remains key.



ECC • Lebih Dari Lubang

Karies, hilang, tampalan gigi susu.

Bawah 72 bulan; tanda awal boleh bermula sebagai white spot.

CLINICAL READING



- Tangkap sebelum cavitation.



Risiko Berkait

Feeding, gula, plak, perlindungan.

ECC multifaktorial; bukan isu
botol susu sahaja.

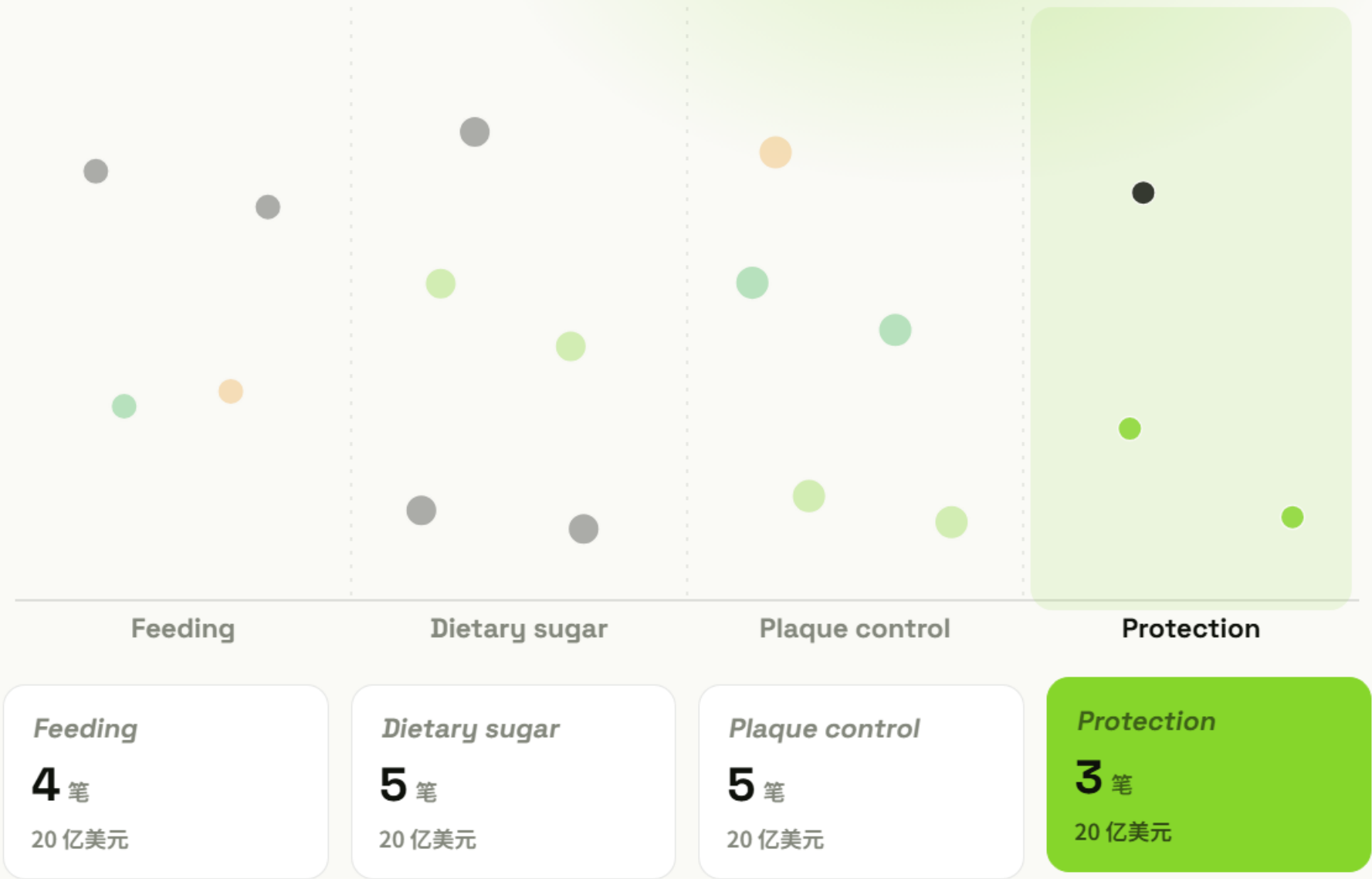
8 risk areas

Use for chairside prompt.

RELATIVE EMPHASIS

- Feeding
- Sugar
- Plaque
- Fluoride

● Risk talk must be practical.



Semak

Ikut Laluan

ASSESS CHAIRSIDE

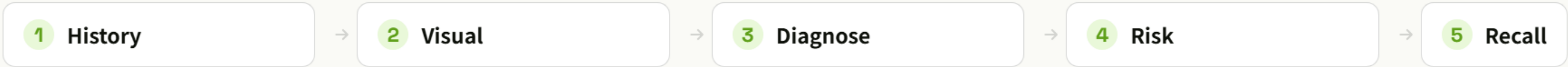
Mulakan dengan sejarah, visual exam dan radiograf.

CRA untuk semua kanak-kanak bawah 6.

CRA
Risk sets recall

STEP	PRIORITY	USE CASE
■ History		5 /100 Feeding, sugar, symptoms.
■ Visual exam		5 /100 Clean, dry, light source.
■ Radiograph		3 /100 Adjunct when indicated.
■ ICDAS		3 /100 Structured scoring aid.
■ CRA		5 /100 Set interval and FV need.

CLINICAL FLOW



- Record, classify, appoint.



Mula Penjaga

Diet, plak dan lawatan berkala.

Sesi penerangan ikut umur, risiko dan keluarga.

● 看好方向

CAREGIVER TALK

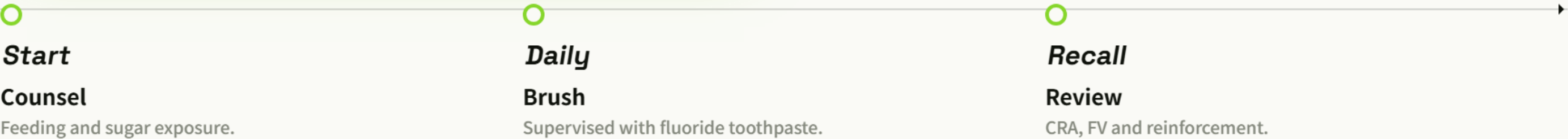
- 01 6-12 bulan
Feeding and oral hygiene talk.
- 02 12-24 bulan
Repeat counselling by risk and need.
- 03 2-6 tahun
Diet, BMI concern and referral.

● 谨慎方向

CLINIC ACTION

- 01 OHE
Mothers, new mothers, carers.
- 02 Plaque control
Mechanical cleaning first.
- 03 FV recall
Risk category sets appointment.

AGE-SPECIFIC EMPHASIS



- Repeat small routines.



Fluoride

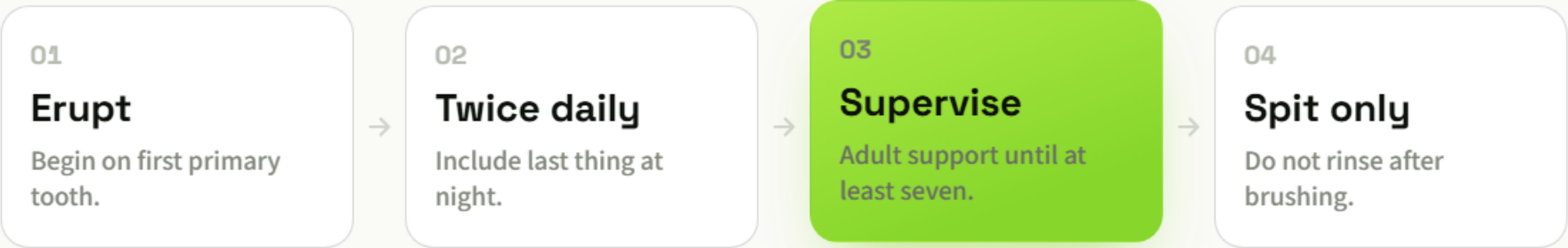
Is Core

HOME CARE

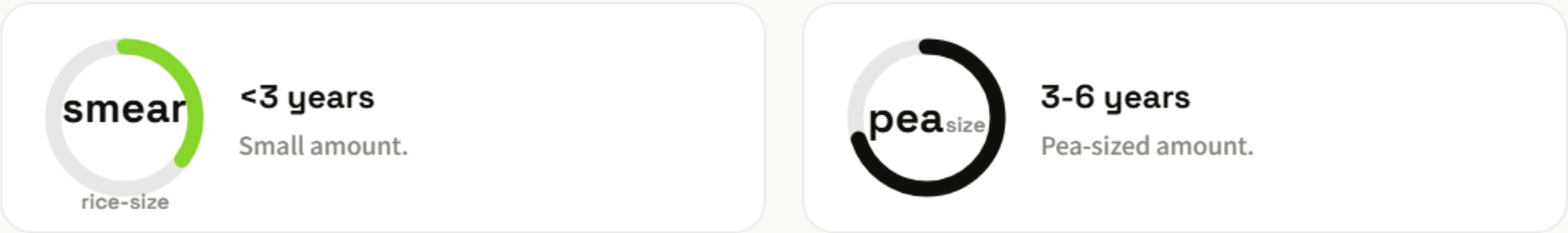
Start brushing when primary teeth erupt; supervise and spit only.

Use 1000-1500 ppm fluoride.

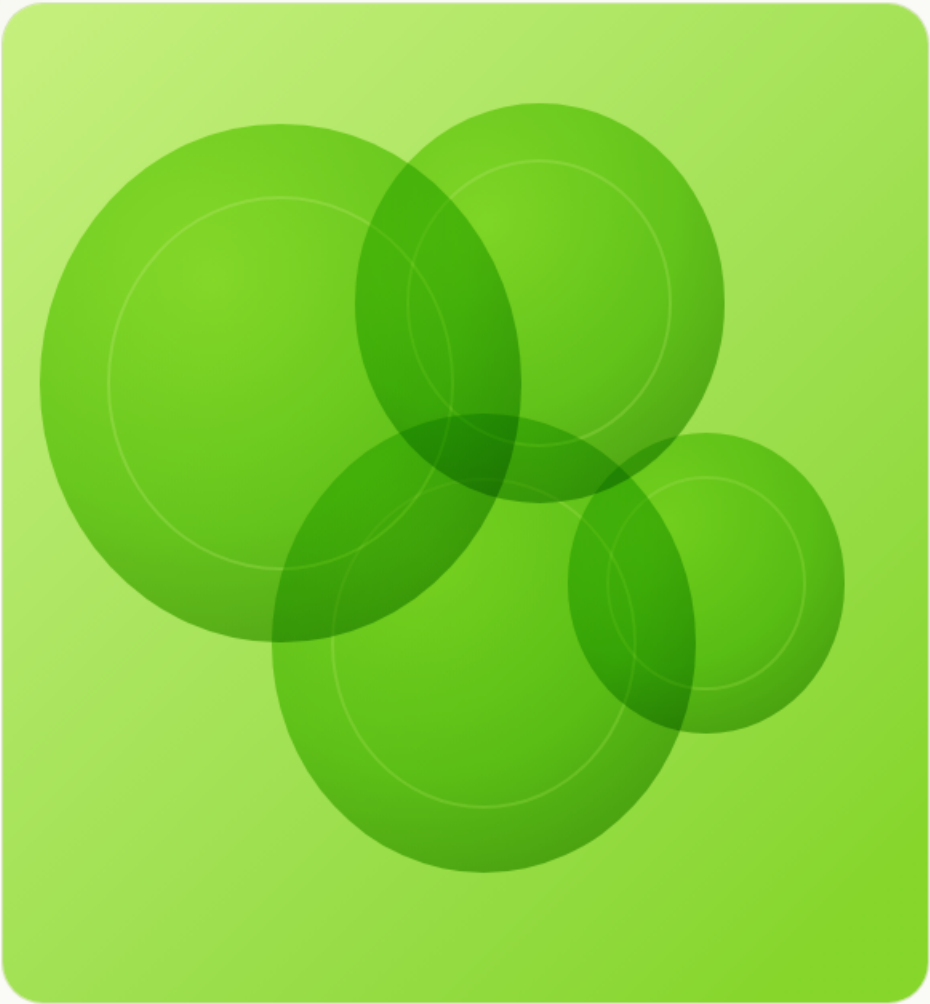
DAILY BRUSHING SCRIPT



DOSE BY AGE



- A simple parent message.



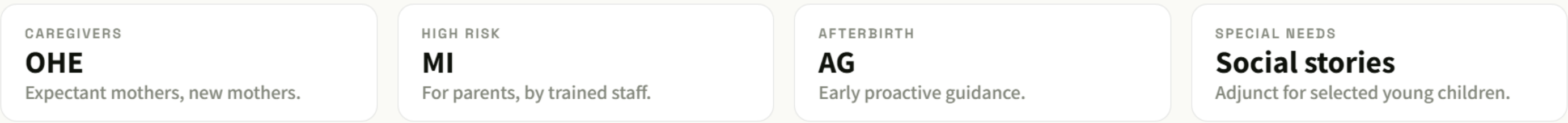
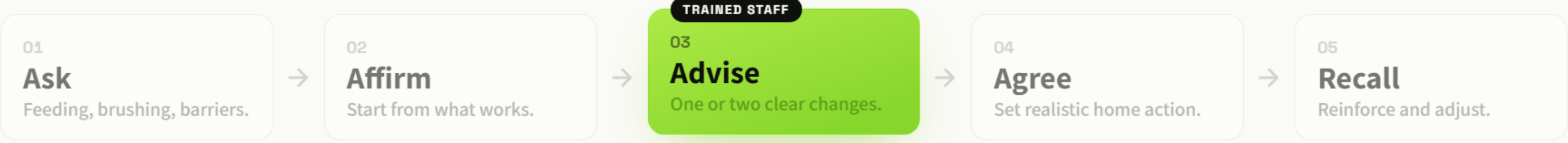
Pendidikan

Perlu Berulang

BEHAVIOUR

OHE, MI and AG support long-term behaviour change.

CONVERSATION MODEL



- REGISTER ANCHORS
- sesi penerangan
 - penjaga
 - risiko tinggi
 - susulan
 - mesra klinik

- Tailor advice.



Bukan Semua

Sama Kuat

PROFESSIONAL CARE

Prioritise fluoride and plaque control before adjunct materials.

DECISION CRITERIA

- 01 **Caries risk**
High risk needs tighter recall.
- 02 **Plaque control**
Home routine must be realistic.
- 03 **Isolation**
Consider before fissure sealant.
- 04 **Cooperation**
Plan safe, staged treatment.

- Use adjuncts carefully.

INTERVENTION NOTES

01FLUORIDE VARNISH

FV

0.9% 3-monthly or 5% 6-monthly.

确定性90%

02PRIMARY MOLAR

Sealant

Consider risk and isolation.

确定性65%

03TOPICAL CREAM

CPP-ACP

No clear added benefit shown.

确定性45%

04SUBSTITUTE

Xylitol

As non-cariogenic sugar substitute.

确定性50%



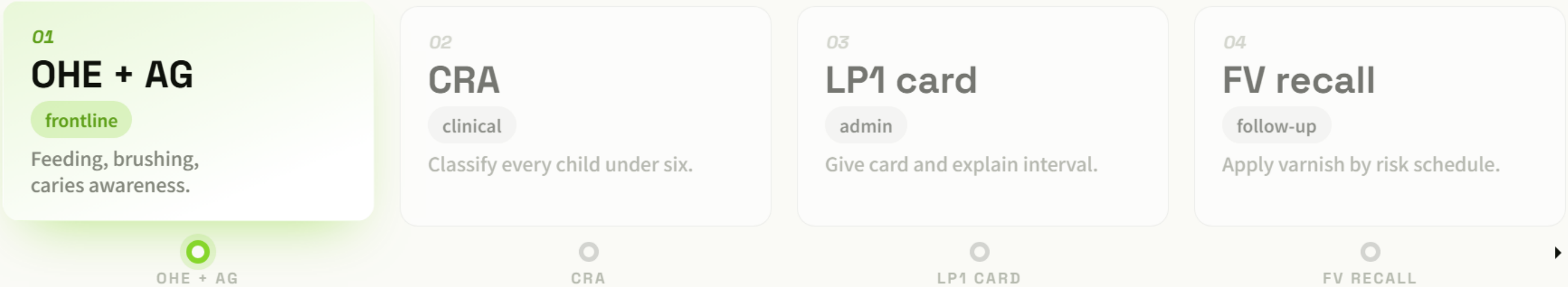
Triage

Then Recall

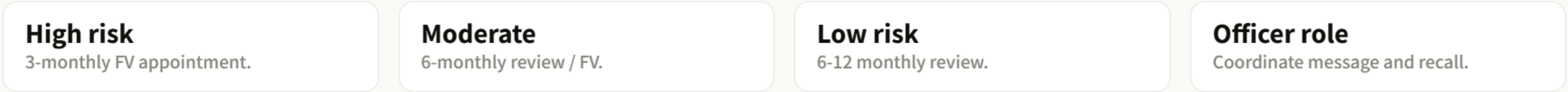
— KKIA / IM COORDINATION

Every child under six gets CRA, appointment card and risk-based follow-up.

CLINIC OPERATING RHYTHM



RECALL INTERVAL



- Risk, interval, action.



— TAKE HOME 06

Ringkas

ECC prevention is a team habit.

Speak clearly, classify risk and repeat prevention.

• OHE • AG • CRA • FV • Recall

• Orderly, friendly.

